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NHS England

Clinical guide for dentistry

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Updates to the previous version of this guide are highlighted in yellow.

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Foreword

In 2015 NHS England published the first of a series of Commissioning Guides and Commissioning Standards, which were produced following publication of Securing excellence in commissioning NHS dental services 2013.

The Commissioning Guides and Standards describe how dental care pathways should develop to deliver consistency and excellence in commissioning NHS dental services across the spectrum of providers to benefit patients.

Over the last 4 years NHS England has taken the opportunity to review the Commissioning Standard programme and reformat for online use, in order to provide a suite of Clinical Standards each linked to an overarching Commissioning Standard. This document is the revised and updated overarching Commissioning Standard which has evolved from the 2015 version.

This Overarching Standard includes associated appendices which will be derived from the current suite of Commissioning Standards and Commissioning Guides which look at specific dental specialties and areas of care. The move to an online platform will allow regular review and revision in line with current policy and practice; it will be necessary to review and update the standards regularly. Implementation will require energy and momentum, together with a willingness to share good practice, innovation and learning to enhance patient care.

1. Equality and health inequalities statement

Promoting equality and addressing health inequalities are at the heart of NHS England's values. Throughout the development of the policies and processes cited in this document, we have: Given due regard to the need to eliminate discrimination, harassment and victimisation, to advance equality of opportunity, and to foster good relations between people who share a relevant protected characteristic (as cited under the Equality Act 2010) and those who do not share it; and, regard to the need to reduce inequalities between patients in access to, and outcomes from, healthcare services and to ensure services are provided in an integrated way where this might reduce health inequalities.

2. Executive summary

NHS England has developed a suite of Clinical Standards for Dental Specialties which intended offer a consistent and coherent qualitative approach for NHS commissioned services. This approach reflects the need and complexity of patient care and the competency of the clinician required to deliver the clinical intervention rather than by the setting within which the care is delivered. Care is delivered via a pathway approach which provides clarity and consistency for patients, the profession and commissioners. There are nationally agreed minimum specifications for each service, including how quality and outcomes are to be measured, which can be enhanced locally.

These standards describe the concept of clinical engagement and leadership to the Integrated Care System (ICS) through Managed Clinical Networks (MCNs) which will work closely with local commissioners.

Needs-led dental specialist care pathways (as in appendix 1) rely on maintaining and ensuring access to effective primary dental care services; particularly for those groups in the population who do not access care routinely or have additional needs. Commissioners who procure services should use these standards to complete needs assessments, set minimum standards and service direction and ensure that proposed outcomes and quality measures are included in service specifications.

The Standards including this overarching Commissioning Standard, should be made available to potential bidders.

3. NHS dental specialties

NHS England commissions all NHS dental services. The benefit of a single commissioner for dentistry is the ability to plan and produce more consistent standards of delivery and better health outcomes for patients across the whole of England.

A strong emphasis on prevention was underlined in the NHS England Long Term Plan and the production of these standards supports NHS England's pledge to improve prevention:

> "It does so while recognising that a comprehensive approach to preventing ill-health also depends on action that only individuals, companies, communities and national government can take to tackle wider threats to health, and ensure health is hardwired into social and economic policy."

These standards are intended to promote consistent quality of specialist dental care provided to patients. Methods to describe population need and current services, working jointly with the National Institute for Health Protection (NIHP), are modelled and shared for commissioners and clinicians to inform local needs assessment and the impact of existing services in meeting identified need. These Standards are about supporting commissioners and clinicians to work together to ensure that resources invested by the NHS in dental specialist care are used in the most effective way to provide the best possible quality and quantity of care for patients.

The levels of complexity do not describe contracts, or practitioners or settings. Levels 1, 2 and 3 care descriptors reflect a case in terms of procedural difficulty, patient modifying factors and competence required of a clinician to deliver care of that level of complexity. Each supporting appendix will detail the requirements of the three levels in regard to that specific clinical requirement.

The three levels of care are defined in table 1 below:

Table 1: Three levels of care

Complexity	Description
Level 1	Skillsets and competencies that are covered by teaching and training in the dental undergraduate and Dental Foundation (FD) programme. Such skills are enhanced and improved with experience, so the development of these skills is a career-long process. The provider is responsible for the delivery and quality of mandatory services. Treatments that are not specifically defined in Levels 2 or 3 are de facto Level 1
Level 2	Procedural and/or patient complexity requiring a clinician with enhanced skills and experience who may or may not be on a specialist register. This care may require additional equipment or environment standards but can usually be provided in primary care. Level 2 case complexity may be delivered as part of the continuing care of a patient or may require onward referral. Providers of Level 2 care on referral will need a formal link to a specialist, to quality assure the outcome of pathway delivery.
Level 3a Level 3b	Care that requires specialist practitioner or consultant led care due to complex clinical or patient factors. This care can be provided in a primary care, dental hospital or in a secondary care setting depending on the needs of the patient and/or local arrangements which may include current training commitments.

These Standards are intended to inform and support Local Dental Networks and commissioners, working within the local context of Primary Care Networks and integrated care systems, to understand local need, impact of current investment and what would enable them to transform services in their area to build on any engagement work already undertaken with local partners such as Healthwatch and patients and the public. There will be a need to establish, strengthen or formalise clinical networks and specialist groups to implement system change which takes into account local needs and circumstances, including reviewing progress to date and required pace of change.